

RECORDS RELEASE — TRANSMITTAL COVER SHEET

TO:	Health Information Management — Records Request
FROM:	MERIDIAN PRIMARY CARE ASSOCIATES, Release of Information Desk
DATE:	02/10/2023
RE:	WHITFIELD, DANA L. DOB 03/14/1991 MRN MPC-118374
PAGES:	12 (including this cover)
AUTHORIZATION:	Signed patient authorization dated 01/27/2023 on file.
CONTENTS:	Intake form; progress notes 09/06/2022 and 10/28/2022; outside eye examination 11/21/2022; neurology co

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MERIDIAN PRIMARY CARE ASSOCIATES — NEW PATIENT INTAKE

Please print clearly. Date: 09/06/2022

Name:	Dana L. Whitfield	DOB:	3/14/91
Address:	88 Concord Lane Apt 3B, Springfield IL 62704	Phone:	(217) 555-0142
Insurance:	BlueCare PPO	Member ID:	XQP448210077
Emergency contact:	Marcus Whitfield (spouse)	Phone:	(217) 555-0177
Reason for visit:	Bad headaches, about 3 weeks		
Current medications:	Depo shot (birth control) since 2021; multivitamin		
Allergies:	None	Pharmacy:	Northgate #204
Past history:	None significant / no surgeries		
Family history:	Mother – migraines; Father – HTN		
Tobacco / Alcohol:	Never / occasional (1-2 drinks per month)		

Patient signature: Dana L. Whitfield Date: 9/6/22

Progress Note — 09/06/2022 (New Patient, Est. Care)**Chief Complaint:** Headaches × 3 weeks.**HPI:** 31-year-old female with ~3 weeks of daily pressure-like headaches, worse in the morning and with bending forward. Occasional pulsatile tinnitus. No fever, no neck stiffness. Reports current use of injectable contraception (Depo-Provera 150 mg/mL IM every 3 months since April 2021, administered at Lakeview Women's Health Clinic; last injection 07/19/2022). Denies head trauma. Denies chest pain. Denies fever or weight loss. No visual changes reported today.**Exam:** BP 124/82, HR 80, BMI 25.1. Neuro grossly intact; fundi not visualized adequately in office. Neck supple.**Assessment:** 1) Headache, new pattern (R51.9). 2) Long-term use of injectable contraceptive.**Plan:** Start ibuprofen 600 mg PO q6h PRN (#60, 2 refills). Headache diary. Return 6–8 weeks or sooner if worsening; advised to report any visual symptoms immediately. Discussed possible association of hormonal contraceptives with headache; will coordinate with GYN if symptoms persist.

Electronically signed: Sofia Grant, DO — 09/06/2022 16:41

Progress Note — 10/28/2022 (Follow-up)

Chief Complaint: Headaches worsening; new visual symptoms.

HPI: Headaches now most days, severity 6–7/10, with brief episodes of dimming vision when standing (seconds, both eyes) over the past 2 weeks — concerning for transient visual obscurations. Ibuprofen partially effective. Deferred her injectable contraceptive dose that was due mid-October pending evaluation. Denies weakness, numbness, or speech difficulty. Denies fever.

Exam: BP 126/84. Visual acuity grossly 20/25 OU with correction. Neuro non-focal.

Assessment: 1) Chronic daily headache with transient visual obscurations — papilledema must be excluded. 2) Injectable contraceptive use — hold pending workup.

Plan: Rx sumatriptan 50 mg PO at headache onset, max 2 tabs/day (#9, 1 refill). Urgent referral to Northgate Eye Associates for dilated fundus examination. If papilledema confirmed, refer neurology. Patient counseled on warning signs; verbalized understanding.

Electronically signed: Sofia Grant, DO — 10/28/2022 11:07

Comprehensive Eye Examination — 11/21/2022

Referral: Dr. Sofia Grant (Meridian Primary Care) — rule out papilledema; chronic headache with transient visual obscurations.

	OD (Right)	OS (Left)
Visual acuity (corrected)	20/25	20/30
Pupils	PERRL, no APD	PERRL, no APD
IOP (mmHg)	16	17
Dilated fundus	Optic disc edema, Frisen grade 2	Optic disc edema, Frisen grade 2; small flame hemorrhage inferior
Visual field (Humphrey 24-2)	Enlarged blind spot	Enlarged blind spot

Assessment: Bilateral papilledema (H47.10) in a 31-year-old female with chronic headaches, on injectable progestin contraception. Findings concerning for elevated intracranial pressure.

Plan: Same-week referral to Springfield Neurology Group (Dr. Natarajan). Recommend neuroimaging (MRI brain with MRV) prior to lumbar puncture. Report faxed to PCP and neurology today.

Electronically signed: Tunde Okafor, OD — 11/21/2022 15:52

Neurology Consultation — 12/09/2022

Reason for consult: Bilateral papilledema, chronic headache, transient visual obscurations, pulsatile tinnitus.

HPI: 31-year-old woman, BMI 25, with ~3 months of progressive daily headaches, worse supine and in the morning, brief bilateral visual dimming with posture change, and whooshing tinnitus. **Medication history is notable for medroxyprogesterone acetate (Depo-Provera) 150 mg IM every 12–13 weeks from 04/13/2021 through 07/19/2022** (six documented injections, per patient and available records). Ibuprofen and sumatriptan provide partial relief. No vision loss episodes lasting > 30 seconds. No diplopia currently.

ROS: Positive: headache, transient visual obscurations, pulsatile tinnitus. Negative: denies diplopia at rest, denies focal weakness, denies seizures, denies fever.

Exam: Alert, oriented. Cranial nerves II–XII intact except fundoscopy showing bilateral optic disc edema consistent with outside report. Motor 5/5 throughout; reflexes 2+ symmetric; gait normal.

Imaging: MRI brain with and without contrast and MRV (12/02/2022, Springfield Imaging Center): no mass, no hydrocephalus, no venous sinus thrombosis. Partially empty sella and posterior globe flattening noted — findings compatible with elevated intracranial pressure.

Neurology Consultation — 12/09/2022 (continued)

Impression: Clinical picture highly suggestive of **idiopathic intracranial hypertension (pseudotumor cerebri, G93.2)**. Literature reports an association between **progestin-only injectable** contraception and intracranial hypertension in some patients; given the **temporal relationship** here, the **injectable contraceptive** is a potentially **contributing factor** and **should be discontinued** in coordination with her gynecologist.

Plan: 1) Diagnostic/therapeutic **lumbar puncture** with **opening pressure**, scheduled 12/16/2022. 2) If confirmed, start **acetazolamide 500 mg PO BID** (dispense as 250 mg tabs, 2 BID) with metabolic panel in 4 weeks. 3) **Recommend discontinuation of Depo-Provera**; letter to Dr. Reyes (Lakeview Women's Health). 4) Ophthalmology follow-up with serial visual fields q6–8 weeks. 5) Weight-neutral counseling; **headache** diary; return 4 weeks post-LP.

Electronically signed: Priya Natarajan, MD — 12/09/2022 17:26

Procedure Note — Lumbar Puncture — 12/16/2022

Indication	Suspected idiopathic intracranial hypertension (G93.2)
Consent	Written informed consent obtained; risks reviewed.
Position / Level	Left lateral decubitus, L3–L4 interspace
Anesthesia	1% lidocaine local
Opening pressure	32 cm H ₂ O (elevated; reference 6–25 cm H ₂ O)
CSF removed	12 mL; closing pressure 18 cm H ₂ O
CSF appearance	Clear, colorless
CSF studies	WBC 1/μL; RBC 0/μL; protein 28 mg/dL; glucose 61 mg/dL — within normal limits
Complications	None; patient tolerated well, discharged ambulatory.

Post-procedure diagnosis: Idiopathic intracranial hypertension, confirmed (opening pressure 32 cm H₂O with normal CSF composition and neuroimaging). Acetazolamide 500 mg PO BID started today; e-script sent to Northgate Pharmacy #204. Headache precautions reviewed.

Electronically signed: Priya Natarajan, MD — 12/16/2022 13:58

MEDICATION ADMINISTRATION RECORD (CONSOLIDATED)

Patient: WHITFIELD, DANA L. DOB: 03/14/1991 Source: Lakeview Women's Health Clinic injection log (copy)

Date	Medication / Dose / Route	NDC	Lot	Site	Administered By	Prescriber
04/13/2021	Depo-Provera (medroxyprogesterone acetate) 150 mg/mL IM	009-0746-30	KJ4471	L deltoid	M. Ortiz, RN	A. Reyes, MD
07/20/2021	Depo-Provera (medroxyprogesterone acetate) 150 mg/mL IM	009-0746-30	KJ4471	R deltoid	M. Ortiz, RN	A. Reyes, MD
10/19/2021	Depo-Provera (medroxyprogesterone acetate) 150 mg/mL IM	009-0746-30	LM0283	L deltoid	C. Boateng, LPN	A. Reyes, MD
01/18/2022	Depo-Provera (medroxyprogesterone acetate) 150 mg/mL IM	009-0746-30	LM0283	R gluteal	M. Ortiz, RN	A. Reyes, MD
04/12/2022	Depo-Provera (medroxyprogesterone acetate) 150 mg/mL IM	009-0746-30	NP7719	L deltoid	C. Boateng, LPN	A. Reyes, MD
07/19/2022	Depo-Provera (medroxyprogesterone acetate) 150 mg/mL IM	009-0746-30	NP7719	R deltoid	M. Ortiz, RN	A. Reyes, MD

Injection due on/about 10/18/2022 — **not administered** (patient deferred; medication later discontinued 01/05/2023 per prescriber note).

BILLING AND CHARGES SUMMARY

Patient: WHITFIELD, DANA L. Guarantor: Self Payer: BlueCare PPO

DOS	Provider / Facility	Code	Description	Charge	Paid	Pt Resp
04/12/2022	Lakeview Women's Health	99213	Office visit, est., low MDM	\$142.00	\$96.40	\$25.00
04/12/2022	Lakeview Women's Health	96372	Therapeutic injection, SC/IM	\$48.00	\$31.10	\$0.00
04/12/2022	Lakeview Women's Health	J1050	Medroxyprogesterone acetate, 150 mg	\$118.00	\$92.75	\$0.00
09/06/2022	Meridian Primary Care	99204	Office visit, new, mod MDM	\$268.00	\$187.60	\$40.00
10/28/2022	Meridian Primary Care	99213	Office visit, est., low MDM	\$142.00	\$96.40	\$25.00
11/21/2022	Northgate Eye Associates	92004	Comprehensive eye exam, new	\$210.00	\$151.30	\$30.00
12/02/2022	Springfield Imaging Center	70551	MRI brain w/o contrast	\$1,486.00	\$1,022.18	\$150.00
12/09/2022	Springfield Neurology Group	99244	Consultation, comprehensive	\$364.00	\$262.90	\$40.00
12/16/2022	Springfield Neurology Group	62270	Spinal puncture, diagnostic	\$412.00	\$298.60	\$50.00

Totals: Charges \$3,190.00 • Insurance paid \$2,239.23 • Patient responsibility \$360.00 • Adjustments \$590.77

LABORATORY REPORT — COMPREHENSIVE METABOLIC PANEL

Collected: 01/13/2023 08:12 Resulted: 01/13/2023 14:47 Ordering: P. Natarajan, MD (acetazolamide monitoring)

Analyte	Result	Flag	Reference
Sodium	138 mmol/L		135–145
Potassium	3.7 mmol/L		3.5–5.1
Chloride	108 mmol/L	H	98–107
CO2 (bicarbonate)	19 mmol/L	L	22–29
BUN	14 mg/dL		7–20
Creatinine	0.82 mg/dL		0.6–1.1
Glucose	91 mg/dL		70–99
Calcium	9.3 mg/dL		8.6–10.2
ALT	21 U/L		7–35
AST	24 U/L		10–40

Interpretation: mild non-anion-gap metabolic acidosis with borderline hyperchloremia, consistent with expected acetazolamide effect; no action required. Continue current dose; repeat CMP in 3 months.

CERTIFICATION OF RECORDS

I certify that the preceding pages are true and complete copies of records maintained in the ordinary course of business for the patient identified above, produced pursuant to the authorization on file.

K. Moreau, Release of Information Specialist
MERIDIAN PRIMARY CARE ASSOCIATES — 02/10/2023

*** End of Packet — 12 pages ***